

MERIDIAN OCCUPATIONAL HEALTH
Initial Evaluation + Treatment Note

Patient: Crawford, Samuel
DOB: 1985-08-12 MRN: MOH-2024-7715
Date of Service: 07/15/2024
Rendering Provider: Helena Marsh, MD (PM&R, NPI 1331100886)
Encounter Type: New patient evaluation (CPT 99203) + procedures

CHIEF COMPLAINT

Right shoulder pain, 5 days, after recreational activity. NOT work-related – see history below.

HPI

39-year-old right-hand-dominant male, employed as a software engineer (sedentary desk-based role, no overhead reaching responsibilities), presents with acute-on-subacute right shoulder pain that began on the evening of 07/10/2024. Mechanism of injury: while playing recreational pickup basketball at a community gym during personal time on a Saturday evening, patient reached overhead aggressively to block a shot, felt an immediate sharp pain in the lateral deltoid/anterior shoulder, and had difficulty raising the arm above shoulder height afterward. Pain is now 5/10 at rest and 8/10 with overhead motion. Reports decreased range of motion and night pain when sleeping on the right side.

OCCUPATIONAL HISTORY (per claim qualification for CO-19 appeal)
Patient confirms – and is willing to sign a separate written attestation – that:

1. The injury occurred OUTSIDE of work hours, off employer premises, during personal recreational activity (community basketball league, not employer-sponsored).
2. Patient's occupation is fully sedentary (software engineering from a desk-based workstation). There is no occupational task that required overhead reaching, lifting, or repetitive shoulder loading at or near the time of injury.
3. No prior occupational claims filed; no prior shoulder injuries on file.
4. Employer has been notified informally; no workers' compensation First Report of Injury has been filed because the injury is not work-related.

This claim should be billed to the patient's commercial health plan, NOT to a workers' compensation carrier. Documentation supports non-occupational etiology.

EXAM

Right shoulder:

- Inspection: no visible deformity or bruising. No muscle atrophy.
- Range of motion: active forward flexion 110° (vs 175° left), abduction 95° (vs 170°), external rotation 40° (vs 80°).
- Strength: 4/5 supraspinatus (empty can), 4-/5 external rotation, 5/5 elbow flexion/extension.
- Special tests: positive Neer impingement sign, positive Hawkins-Kennedy, positive empty-can. Lift-off test difficult to perform due to pain.
- Distal neurovascular intact.

ASSESSMENT

1. Acute rotator cuff strain, right shoulder, non-occupational (S46.011A) – recreational injury.

2. Shoulder impingement syndrome, right (M75.41) – likely secondary process.

PLAN – TODAY'S ENCOUNTER & PROCEDURES

1. CPT 99203 – initial evaluation completed as above.
2. CPT 97140 – Manual therapy techniques (myofascial release of trapezius, posterior capsular stretch, scapular mobilization). Performed in-clinic by PT extender under MD supervision; 15 min one-on-one.
3. CPT 97110 – Therapeutic exercise (scapular stabilization, pendulum, isometric external rotation). 30 min one-on-one.
4. Ice and NSAID counseling.
5. Sling provided for comfort, weaning over 5-7 days.
6. RTC 1 week for re-evaluation; consider MRI if persistent weakness or no improvement.

ATTESTATION

Patient verbally confirmed at the start of the visit, and this provider has documented, that the injury did not occur during the course of employment and that this care should not be billed to workers' compensation.

Electronically signed: Marsh, Helena MD on 07/15/2024 15:09